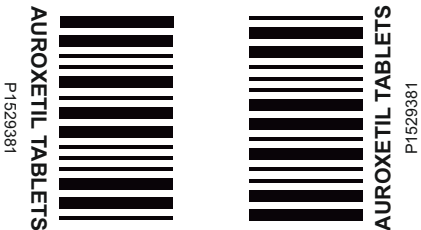


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SUMMARY OF PRODUCT CHARACTERISTICS

AUROXETIL

Cefuroxime Axetil Tablets
125 mg, 250 mg and 500 mg

Rx Only

NAME OF DRUG PRODUCT: Cefuroxime Axetil Tablets 125 mg.
Cefuroxime Axetil Tablets 250 mg.
Cefuroxime Axetil Tablets 500 mg.

(TRADE) NAME OF PRODUCT : AUROXETIL 125.
AUROXETIL 250.
AUROXETIL 500.

STRENGTH: 125 mg, 250 mg and 500 mg.

PHARMACEUTICAL DOSAGE FORM:Tablet.

QUALITATIVE AND QUANTITATIVE COMPOSITIONS:

Cefuroxime Axetil Tablets 125 mg

Each uncoated tablet contains:

Cefuroxime Axetil equivalent to Cefuroxime 125 mg.

Cefuroxime Axetil Tablets 250 mg

Each uncoated tablet contains:

Cefuroxime Axetil equivalent to Cefuroxime 250 mg.

Cefuroxime Axetil Tablets 500 mg

Each uncoated tablet contains:

Cefuroxime Axetil equivalent to Cefuroxime 500 mg.

PHARMACEUTICAL FORM

Cefuroxime Axetil Tablets 125 mg:

White to off-white, uncoated, capsule shaped tablets with 'A32' debossed on one side and plain on the other side.

Cefuroxime Axetil Tablets 250 mg:

White to off-white, uncoated, capsule shaped tablets with 'A33' debossed on one side and plain on the other side.

Cefuroxime Axetil Tablets 500 mg:

White to off-white, uncoated, capsule shaped tablets with 'A34' debossed on one side and plain on the other side.

CLINICAL PARTICULARS

Pharmacodynamics/Pharmacokinetics

Pharmacodynamic:

Mechanism of Action

Cefuroxime Axetil is an oral prodrug of the bactericidal cephalosporin antibiotic cefuroxime, which is resistant to most β-lactamases and is active against a wide range of gram-positive and gram-negative organisms. It is indicated for the treatment of infections caused by sensitive bacteria.

Microbiology: Cefuroxime axetil owes its *in vivo* bactericidal activity to the parent compound, cefuroxime. Cefuroxime is a well-characterised and effective antibacterial agent which has bactericidal activity against a wide range of common pathogens, including β-lactamase-producing strains. Cefuroxime has good stability to bacterial β-lactamase and consequently, is active against many ampicillin-resistant or amoxycillin-resistant strains. The bactericidal action of Cefuroxime results from inhibition of cell wall synthesis by binding to essential target proteins.

Cefuroxime is usually active against the following organisms *in vitro*: Aerobes: Gram-Negative: *Haemophilus influenzae* (including ampicillin-resistant strains); *Haemophilus parainfluenzae*; *Moraxella (Branhamella) catarrhalis*; *Neisseria gonorrhoeae* (including penicillinase- and non-penicillinase-producing strains); *Escherichia coli*; *Klebsiella sp*; *Proteus mirabilis*; *Providencia sp*; *Proteus rettgeri*.

Aerobes: Gram-Positive: *Staphylococcus aureus* and *Staphylococcus epidermidis* (including penicillinase-producing strains but excluding methicillin-resistant strains), *Streptococcus pyogenes* (and other β-haemolytic streptococci), *Streptococcus pneumoniae*, Streptococcus Group B (*Streptococcus agalactiae*).

Anaerobes: Gram-positive and gram-negative cocci (including *Peptococcus sp* and *Peptostreptococcus sp*); gram-positive bacilli (including *Clostridium sp*) and gram-negative bacilli (including *Bacteroides sp* and *Fusobacterium sp*); *Propionibacterium sp*.

Other Organisms: *Borrelia burgdorferi*.

The following organisms are not susceptible to cefuroxime: *Clostridium difficile*, *Pseudomonas sp*, and *Campylobacter sp*, *Acinetobacter calcoaceticus*, *Listeria monocytogenes*, Methicillin-resistant strains of *Staphylococcus aureus* and *Staphylococcus epidermidis*, *Legionella sp*.

Some strains of the following genera are not susceptible to cefuroxime: *Enterococcus (Streptococcus) faecalis*, *Morganella morganii*, *Proteus vulgaris*, *Enterobacter sp*, *Citrobacter sp* and *Serratia sp*, *Bacteroides fragilis*.

Pharmacokinetics:

After oral administration, Cefuroxime axetil is slowly absorbed from the gastrointestinal

tract and rapidly hydrolysed in the intestinal mucosa and blood to release Cefuroxime into the circulation.

Following administration of Cefuroxime axetil tablets, peak serum levels (2.9 mg/L for a 125-mg dose, 4.4 mg/L for a 250-mg dose, 7.7 mg/L for a 500-mg dose and 13.6 mg/L for a 1-g dose) occur approximately 2.4 hrs after dosing when taken with food.

Optimum absorption occurs when it is administered after a meal.

Peak serum levels (2-3 mg/L for a 125-mg dose, 4-6 mg/L for a 250-mg dose, 5-8 mg/L for a 500-mg dose and 9-14 mg/L for a 1-g dose) occur approximately 2-3 hrs after dosing when taken after food, unlike IV dosing which peaks immediately.

Post peak levels, the serum half-life is between 1 and 1.5 hrs. Protein-binding has been variously stated as 33-50% depending on the methodology used. Cefuroxime is not metabolised and is excreted by glomerular filtration and tubular secretion.

Concurrent administration of probenecid increases the area under the mean serum concentration-time curve (AUC) by 50%.

Serum levels of Cefuroxime are reduced by dialysis.

Toxicology: Preclinical Safety Data: Animal toxicity studies indicated that Cefuroxime axetil is of low toxicity with no significant findings.

Indication:

Cefuroxime Axetil Tablets are indicated for the treatment of patients with mild to moderate infections caused by susceptible strains of the designated microorganisms in the conditions listed below.

Upper respiratory tract infections eg, ear, nose and throat infections ie, otitis media, sinusitis, tonsillitis and pharyngitis.

Lower respiratory tract infections eg, pneumonia, acute bronchitis and acute exacerbations of chronic bronchitis.

Genitourinary tract infections eg, pyelonephritis, cystitis and urethritis.

Skin and soft-tissue infections eg, furunculosis, pyoderma and impetigo.

Gonorrhoea, acute uncomplicated gonococcal urethritis and cervicitis.

Cefuroxime is also available as the sodium salt (Cefuroxime Sodium) for parenteral administration. This permits the use of sequential therapy with the same antibiotic, when a change from parenteral to oral therapy is clinically indicated.

Where appropriate, Cefuroxime Axetil is effective when used following initial parenteral Cefuroxime Sodium in the treatment of pneumonia and acute exacerbations of chronic bronchitis.

Recommended Dose

Cefuroxime Axetil Tablets (May be administered without regard to meals).

The usual course of therapy is 7 days (Range 5-10 days).

Cefuroxime Axetil should be taken after food for optimum absorption.

Adults: Most Infections: 250 mg twice daily.

Urinary Tract Infections: 125 mg twice daily.

Mild to Moderate Lower Tract Infections eg, Bronchitis: 250 mg twice daily.

More Severe Lower Tract Infections (or if pneumonia is suspected): 500 mg twice daily.

Pyelonephritis: 250 mg twice daily.

Uncomplicated Gonorrhoea: Single dose of 1 g.

Sequential Therapy: **Pneumonia:** Cefuroxime Sodium 1.5 g 2-3 times a day (IV or IM) for 48-72 hrs followed by Cefuroxime Axetil (Cefuroxime axetil) 500 mg twice a day oral therapy for 7-10 days.

Acute Exacerbations of Chronic Bronchitis: Cefuroxime Sodium 750 mg 2-3 times a day (IV or IM) for 48-72 hrs, followed by Cefuroxime Axetil 500 mg twice a day oral therapy for 5-10 days.

Duration of both parenteral and oral therapy is determined by the severity of the infection and the clinical status of the patient.

Children: Most Infections: 125 mg twice daily, to a maximum of 250 mg daily.

Children ≥ 2 years: Otitis Media or Where Appropriate, with More Severe Infections: 250 mg twice daily, to a maximum of 500 mg daily.

Cefuroxime Axetil tablets should not be crushed and are therefore, unsuitable for treatment of patients eg, younger children, who cannot swallow tablets. In children, Cefuroxime Axetil oral suspension may be used.

There is no experience of using Cefuroxime Axetil in children <3 months.

Mode of Administration

Oral route

Should be taken with food (Take immediately after food).

Contraindications

Hypersensitivity to cephalosporins.

Warning and Precautions

Special care is indicated in patients who have experienced an allergic reaction to penicillins or other β-lactams.

Serious and occasionally fatal hypersensitivity reactions (including anaphylactoid and severe cutaneous adverse reactions) have been reported in patients receiving therapy with β - lactams. Before initiating therapy with cefuroxime axetil, careful inquiry should be

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Malaysia_Healol Pharma	01	Revision			
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Initiator	Shirisha	210 x 300 mm			
Artist: SCD		Pharmacode		 29381	
		29381			
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					Sign / Date

made concerning previous hypersensitivity reactions to penicillins, cephalosporins, carbapenems or other β – lactam agents. If an allergic reaction occurs, cefuroxime axetil must be discontinued immediately and appropriate alternative therapy instituted.

As with other antibiotics, prolonged use of Cefuroxime axetil may result in the overgrowth of nonsusceptible organisms (eg, Candida, Enterococci, Clostridium difficile), which may require interruption of treatment.

Pseudomembranous colitis has been reported with the use of broad-spectrum antibiotics, therefore, it is important to consider its diagnosis in patients who develop serious diarrhoea during or after antibiotic use.

With a sequential therapy regimen, the timing of change to oral therapy is determined by the severity of the infection, clinical status of the patient and susceptibility of the pathogens involved. If there is no clinical improvement within 72 hrs, then the parenteral course of treatment must be continued.

Effects on Ability to Drive and Use Machine

As this medicine may cause dizziness, patients should be warned to be cautious when driving or operating machinery.

Use in pregnancy & lactation: There is no experimental evidence of embryopathic or teratogenic effects attributable to Cefuroxime axetil but, as with all drugs, it should be administered with caution during the early months of pregnancy. Cefuroxime is excreted in human milk and consequently, caution should be exercised when Cefuroxime Axetil is administered to a nursing mother.

Interaction with other Medicaments

Drugs which reduce gastric acidity may result in a lower bioavailability of Cefuroxime Axetil compared with that of the fasting state and tend to cancel the effect of enhanced postprandial absorption.

As a false-negative result may occur in the ferricyanide test, it is recommended that either the glucose oxidase or hexokinase methods are used to determine blood/plasma glucose levels in patients receiving Cefuroxime axetil. This antibiotic does not interfere in the alkaline picrate assay for creatinine.

Pregnancy and Lactation

Pregnancy:

A	B	C	D	X
	▲			

Category B: Either animal-reproduction studies have not demonstrated a foetal risk but there are no controlled studies in pregnant women or animal-reproduction studies have shown an adverse effect (other than a decrease in fertility) that was not confirmed in controlled studies in women in the 1st trimester (and there is no evidence of a risk in later trimesters).

Lactation:

Cefuroxime is excreted in human milk, consideration should be given to discontinuing nursing temporarily during treatment with Cefuroxime Axetil.

Side Effects

Adverse reactions to Cefuroxime axetil have been generally mild and transient in nature.

As with other cephalosporins, there have been rare reports of erythema multiforme, Stevens-Johnson syndrome, toxic epidermal necrolysis (exanthematic necrolysis) and hypersensitivity reactions including skin rashes, urticaria, pruritus, drug fever, serum sickness, and very rarely, anaphylaxis.

A small proportion of patients receiving Cefuroxime Axetil have experienced gastrointestinal disturbances, including diarrhoea, nausea and vomiting. As with other broad-spectrum antibiotics, there have been reports of pseudomembranous colitis.

Headache has also been reported.

Eosinophilia and transient increases of hepatic enzyme levels, [ALT (SGPT), AST (SGOT) and LDH] have been noted during Cefuroxime Axetil therapy.

There have been rare reports of thrombocytopenia and leucopenia (sometimes profound). As with other cephalosporins, jaundice has been reported very rarely.

Cephalosporins as a class tend to be absorbed onto the surface of red cells membranes and react with antibodies directed against the drug to produce a positive Coombs' test (which can interfere with cross-matching of blood) and very rarely, haemolytic anaemia.

Symptoms and Treatment of Overdose

Overdosage of cephalosporins can cause cerebral irritations leading to convulsions.

Serum levels of Cefuroxime can be reduced by haemodialysis or peritoneal dialysis.

PHARMACEUTICAL PARTICULARS

List of Excipients

Cellulose, Microcrystalline, Sodium Laurilsulfate, Croscar-mellose Sodium, Silica, Colloidal Anhydrous and Hydrogenated Vegetable Oil.

Incompatibilities

A positive Coombs' test may occur during treatment with cephalosporins this phenomenon can interfere with cross matching of blood.

Shelf life

Tablet: 3 years.

Storage Condition

Store below 30°C.

Nature and contents of container

Blister Pack:

AUROXETIL 125,AUROXETIL 250 andAUROXETIL 500: 5x10's

Manufactured by



AUROBINDO

Aurobindo Pharma Ltd,

Unit VI, Survey No. 329/39 & 329/47,Chitkul (V), Patancheru Mandal, Sanga Reddy District, Telangana State, India.

Regd. Office: Plot No.: 2, Maitrivihar, Ameerpet, Hyderabad-500 038, Telangana State, India.

Product Registration Holder in Malaysia:

Healol Pharmaceuticals Sdn Bhd, 74-3, Jalan Wangsa Delima 6, KLSC Wangsa Maju, 53300 Kuala Lumpur.

Date of revision: November 2021